

Beyond Culture: Centering Power, Reciprocity, and Justice in HCI and Mental Health Research

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Abstract

Mental health concerns are globally widespread, with most individuals in need never receiving care. Researchers in Human-Computer Interaction (HCI) have investigated how technology might connect people to care, emphasizing that mental health technologies must be considerate of structural factors (such as culture, power, and justice) for digital interventions to be acceptable and effective. In this Special Interest Group (SIG), we convene researchers at the intersection of HCI and mental health to engage in collaborative ideation around how mental health technologies could tangibly integrate considerations of power, reciprocity, and justice. Through guided discussions, we will consider who digital mental health technologies are built for, what users immediately get from HCI and mental health research, and how technologies can balance being personalized while being scalable. We hope to lay the foundation for concrete guidelines around how technology and mental health researchers can be considerate of structural and cultural factors.

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CCS Concepts

• **Human-centered computing** → **Human computer interaction (HCI)**.

Keywords

culture, digital mental health, power, reciprocity, justice

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1 Background and Motivation

Researchers at the intersection of Human-Computer Interaction (HCI) and mental health are increasingly focused on research questions that examine whether technology may help provide mental health support to people in dire need of care. Mental health needs are widespread—half of all people globally experience a mental health disorder during their lives [10], and most do not receive any type of treatment, largely due to the cost of care, stigma, and the debilitating nature of the symptoms of mental illness [34]. Towards making care more accessible, research in HCI and mental health has demonstrated how social media data could be used to deliver

targeted interventions for individuals experiencing rapidly worsening mental health [1], tested the acceptability of varied intervention designs [8, 14, 16, 30], and analyzed how people use popular technologies (such as helplines [19, 21] or search engines [9, 13]) when in states of distress.

Recent work in HCI has also emphasized the critical role that *cultural* considerations play in delivering digital mental health tools. For example, Pendse et al. [20] describe how designers of digital mental health tools often ground designs in Western constructs around mental health, which can exclude culturally-bounded expressions of distress in minoritized groups. Recognizing this core importance of culture and context, HCI researchers have studied the role of military culture in mental health needs [5], the mental health needs of undocumented migrant youth [32], the role of technology in the unique mental health challenges experienced by individuals living in conflict zones [2, 35], how multiple and intersecting forms of marginalization can influence the use of mental health technologies [15], and how the experience of minoritization within a majority culture can shape mental health technology use [18, 29]. This subfield of HCI and mental health research collectively emphasizes the importance of considering culture in how mental health technology is designed, from the scales used to measure mental health within a technology [20] to the interface associated with the technology itself [5, 15]. Culture's core importance may be clear to the designer, HCI researcher, or practitioner newly exploring research topics in digital mental health. However, culture can be complex, and consideration of *structural* factors may be crucial to create sustainable change. For example, in his writing on culture's role in mental health experiences, Kleinman describes how "economic, political, [and] institutional" forces play a core role in how "individuals of unequal power [attempt] to persuade others of the intensity of their distress, and of the need for access to more resources" [7]. Similar to culture, while the importance of these additional structural factors may also be clear, integrating cultural considerations into technology design or study methodologies is still an open question. We propose this Special Interest Group (SIG) session as a beginning towards answering that broader question, or what the tangible integration of considerations of culture in digital mental health applications might look like, particularly given the complex individual and societal factors that intersect with culture.

As organizers, by convening a global group of HCI researchers working at the intersection of culture and mental health, we hope to move the conversation around culture forward by asking what it might look like to *tangibly* integrate structural factors (including power, reciprocity, and justice) into how we conduct HCI and mental health research and design mental health technologies. This SIG thus aims to answer the following questions: **what do mental health technologies that integrate considerations of culture and power tangibly look like? How do modes of interaction, features, and modalities of mental health support change when we center the structural aspects of mental health experience in mental health application design?** Answering such questions necessitates conversations that span the interdisciplinary group of people who create and design digital mental health technologies, including implementation scientists and clinicians who regularly tackle questions around access, real-world acceptability, and unexpected cultural or structural constraints [3, 22, 25, 26].

In the 75-minute SIG discussion session at CHI 2025, we will convene a group of researchers conducting research at the intersection of culture, HCI, and mental health towards mutual learning across fields and the foundation for guidelines around how HCI and mental health researchers and application designers may consider culture in their research and practice. We build on past discussions at CHI that focused on contemporary trends and future directions for research in technology design, mental health, and culture. This includes SIGs centered on interdisciplinary health and HCI research [11, 28, 36], as well as SIGs centered on cultural and structural considerations [4, 33]. We plan to do this through small group discussions centered around the following topics:

- **Centering Class and Power in HCI and Mental Health:** Who are mental health technologies built for? Who are they not built for? How do we close this gap? And when may technology design not be an appropriate approach or result in unintended harm?
- **Reciprocity and Justice in HCI and Mental Health:** What can marginalized people with lived experience *immediately* get from participating in HCI and mental health research? How can we orient our methods towards meeting unique (and global) needs more immediately?
- **Cultural Dimensions of Interaction in Digital Mental Health Interventions:** What aspects of mental health technology ecosystems require attention to culture and identity? What interface design considerations are necessary when designing digital mental health tools for marginalized communities? What are the tensions between personalization and scalability in mental health intervention design?

2 SIG Goals

Through this SIG session, building on past interdisciplinary health SIGs [28], we aim to reflect on past and current HCI and mental health research across disciplines and extend the conversation to identify tangible next steps to implement culturally sensitive mental health technologies. We will explore questions that will challenge attendees' existing knowledge of the boundaries of mental health technologies. Attendees will be asked to think critically about each step of the HCI and mental health research and technology design process.

The SIG will begin with a brief presentation on the existing landscape of digital mental health within HCI research and beyond from the organizing team. This presentation will be followed by three sets of separate group discussions with guiding questions that allow participants to learn about mutual research areas through discussion of power, reciprocity, and justice in current work. Attendees are encouraged to share their perspectives and core issues that they have encountered when considering aspects of power, reciprocity, and justice in their research and practice. The discussions will be facilitated under three major topics:

2.1 Centering Class and Power in HCI and Mental Health:

Factors such as racial discrimination, class divisions, and the digital divide all play a role in individuals' mental health and access to resources. However, most mental health research, including that

at the intersection of HCI and mental health, is conducted with individuals who already have access to digital tools and are likely to have mental health resources [31]. While some technologies are designed with marginalized and vulnerable populations in mind [6], it is unclear when technology-based approaches may be helpful and when they may actually result in unintended harm (by funneling away resources from needed social programs, for example [12]). With this in mind, in this small group discussion, we will discuss how class and power influence both individual experiences with mental health *and* interactions with technology. We hope to collaboratively discuss approaches to ensuring that mental health technologies are acceptable and effective beyond considerations of culture.

2.2 Reciprocity and Justice in HCI and Mental Health

One core aspect of research on mental illness is the sharing of experiences of intimate and personal experiences distress with researchers. However, in many cases, it is unclear what reciprocal benefit participants receive after sharing distressing experiences [27]. Given that many with mental illness come from marginalized groups, researchers in HCI and mental health [20, 21] have framed the need to provide tangible benefit to participants through research design as an important consideration related to justice. This panel will discuss how justice is discussed in existing mental health services and access and will explore how it extends to research in HCI and mental health. Drawing on work from implementation scientists who have tangled with the issue of how to provide benefit through study design [24], in this small group discussion, we will discuss how HCI research methodologies can also center tangible benefit to participants. We hope to discuss concrete approaches to integrate tangible benefits in research methodologies and considerations of justice within HCI and mental health study design.

2.3 Cultural Dimensions of Interaction in Digital Mental Health Interventions

Designing culturally robust mental health technologies involves ensuring such technologies fit within a user's cultural context and needs. One approach to doing so has been to increase culturally relevant components in mental health technology interface designs, such as translating the user interface and varying the avatars' appearances in mental health applications to match those of the users [6, 23]. However, as Pendse et al. [20] note, considerations of culture can influence interactions on more implicit levels, such as through the scales used to measure success of a digital intervention. There is also an additional tension between creating interventions that scale to the most people, and creating interventions that are designed for specific populations (but may not scale as easily). In this small group discussion, we will discuss concrete experiences with integrating culture into digital interventions in a scalable format, including the tension between personalization and scalability.

3 Expected Outcomes and Post-SIG Plans

In line with past SIGs, we will build on the discussions at this SIG towards the creation of a set of concrete guidelines for researchers and designers to incorporate considerations of power, reciprocity,

and justice in research methods and technology design. Attendees will be encouraged to contribute post-SIG if they are interested, with interest gauged via a Google Form circulated at the end of the session. Guidelines will later be shared (with feedback solicited) widely among researchers, practitioners, startup founders, designers, and clinicians through publications, conferences, and blog posts. A Slack channel may be created to provide an ongoing opportunity and a welcoming space for attendees to share insights and strategies in future research and product development for studying (i.e., research approaches) and designing mental health technologies that consider power, reciprocity, and justice.

4 SIG Accessibility

Our organizing team includes organizers with lived experience with chronic mental and physical illness and understand the importance of ensuring that the SIG is as accessible as possible. Prior to the SIG, we will ensure the meet-up is as accessible as possible, which may include having automated transcription during the presentation, providing materials in multiple formats (e.g., digital and printed copies), ensuring wheelchair-accessible spaces, and offering directions to quiet rooms for attendees who need to step away.

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5 Supplementary Materials

5.1 Communities of Interest and Attendee Backgrounds

The primary community of interest for this SIG will be researchers who work at the intersection of culture, mental health, and HCI. However, building on past interdisciplinary SIGs [28], we will invite scholars from fields beyond HCI and mental health, including social work, clinical psychology, medicine, public health, and implementation science. However, we also expect that people who are more broadly interested in questions centered around justice or questions centered around culture may also participate. Following Pendse et al.'s [17] call for mental health to be a constant consideration, including in studies not explicitly centered around mental health, we welcome these communities and are excited about what they can contribute to the discussions. We thus welcome attendees with varying experiences, including junior and senior researchers, aspiring or practicing clinicians, and professionals from the industry. We hope to attract individuals from global backgrounds, particularly given our workshop focus.

5.2 SIG Format and Activities

The SIG will primarily rely on the format of small group discussions. Prior to the SIG, the organizers will prepare a shared Google Doc that includes guiding questions and provides space for note-taking during the small group discussions. During the 75-minute on the day of the SIG meet-up, attendees will organize themselves into small groups that consist of 3 to 5 attendees each. The small group format ensures attendees have the opportunity and time to share their perspectives and experiences. Attendees will be asked to take notes of their discussions on the Google Doc for each of the three topics currently planned for the SIG. After engaging in small group discussions, the lead organizers will facilitate the large group discussion and take notes on key share-outs from each group, using the Google Doc as a guide. After the SIG, attendees are encouraged to exchange contact information to stay connected to each other, with the hope of pioneering new approaches to HCI and mental health research and digital interventions.

5.3 SIG Schedule

The SIG session will begin with a 10-minute introduction, during which lead organizers will present an overview of the goals of the SIG and provide context on the landscape of HCI research at the intersection of mental health, technology, and culture (as discussed in the Background and Motivation section of the main text). Following this introduction, participants will spend 5 minutes introducing themselves within their small groups. The next 45 minutes of the SIG session will be dedicated to three distinct small group discussion periods (each 15 minutes). These discussions will focus on the three themes we outline in the main text—centering class and power in HCI and mental health research, reciprocity and justice in the design of digital interventions, and the cultural dimensions of user interaction in digital mental health interventions. The session will conclude with a 15-minute large group share-out and debrief.

5.4 Call for Participation

We expect to have anywhere between 25 to 30 attendees. We will promote the SIG on social media channels, email lists, and other online and offline forms of publicity within our individual institutions.

5.5 Primary Contact

Sachin Pendse (sachin.r.pendse@northwestern.edu) will be the primary contact for this SIG proposal.